

Assess	When to assess	Note
Syphilis	If not done or not repeated later in pregnancy	Ideally, use rapid fingerprick test, as result immediately available. If HIV negative or unknown, use a dual HIV syphilis rapid test, if available.
HIV test in mother	- At delivery (if not done/repeated) - If breastfeeding: at 10 weeks, 6 months, then 3 monthly	- Test for HIV ↗ 110. If test negative, consider need for PrEP ↗ 106. - If HIV positive, give routine HIV care and start ART same day ↗ 113. Test baby for HIV same day and if breastfeeding, give infant prophylaxis ↗ 166.
Viral load (VL) if HIV positive	- At delivery 6 months after delivery - If breastfeeding: 6 monthly	- Follow up results of VL done at delivery at the 3-6 days postnatal visit. If VL not done at delivery, do at this visit. - If VL < 50, continue ART and give routine HIV care ↗ 111. - If VL ≥ 50: manage unsuppressed VL ↗ 166.
HIV test in baby	- HIV-exposed: birth, 10 weeks, 6 months, 18 months, 6 weeks after breastfeeding stopped - HIV-unexposed: 18 months - At any time if baby unwell	- If mother diagnosed with HIV while breastfeeding or baby unwell, do HIV test same day. - If baby < 18 months old: use HIV PCR as initial test. If positive, start ART and confirm result with second HIV PCR (or HIV viral load). - If baby 18-24 months old: use rapid HIV test as initial test. If positive, confirm with HIV PCR test before starting ART. - If baby ≥ 24 months old: as for adult testing ↗ 110.
Haemoglobin	6 weeks	Give iron according to Hb (see below). If Hb < 10: repeat monthly until Hb reaches 10. If no improvement 1 month after starting treatment, discuss/refer.
Cervical screen	From 6 weeks	- HIV negative: do cervical screen if ≥ 30 years and no screen in past 10 years ↗ 55. - HIV positive: do cervical screen at HIV diagnosis (regardless of age), then 3-yearly ↗ 55.
Rhesus (Rh)	If mother rhesus negative: 6 hour and 6 day visit	If baby rhesus positive/unknown, give mother single dose anti-D immunoglobulin 100mcg IM, preferably within 72 hours, up to 7 days after delivery.

Health for All

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Advise the mother

- Encourage mother to become active soon after delivery, rest frequently and eat well. Advise mother to keep perineum clean and to change pads 4-6 hourly.
- Advise to return urgently if heavy bleeding, foul-smelling vaginal discharge, red/oozing wound, fever, dizziness, severe headache or abdominal pain, blurred vision, calf pain or baby unwell.
- Refer to an infant feeding support group. Give feeding advice:
 - Regardless of HIV status, encourage exclusive breastfeeding for 6 months: baby gets only breast milk (no formula, water, cereal) and if HIV-exposed, infant prophylaxis.
 - Advise to only introduce food from 6 months of age while continuing with feeding choice. Advise the working mother to consider expressing breast milk for baby while away.
 - Regardless of HIV status, continue to breastfeed until 2 years of age. If HIV positive, ensure viral suppression on ART. If HIV negative, advise 3-monthly HIV tests.
 - If mother chooses to formula feed: check that she is able mix it correctly, afford it long-term, has access to clean boiled water, and that it will be acceptable (i.e. no disclosure issues).
- Discuss family planning and importance of spacing children. Advise to use reliable contraception and condoms as soon after delivery as possible.
- Explain that the first 1000 days of a child's life are vital to his/her development: encourage mother and father to respond when baby cries and to hold, talk/sing and make eye contact with baby to help with bonding and development. If mother finds this difficult, encourage her to return more frequently and refer to support group, if available.

Treat the mother

- Give iron¹ according to Hb:
 - If Hb ≥ 10, give **ferrous sulphate compound BPC** 170mg daily or **ferrous fumarate** 200mg daily for 6 weeks after delivery.
 - If daily iron not tolerated², give instead **ferrous sulphate compound BPC** 340mg once weekly with food or **ferrous fumarate** 400mg once weekly with food for 6 weeks.
 - If Hb < 10, give **ferrous sulphate compound BPC** 170mg 12 hourly with food or **ferrous fumarate** 200mg 12 hourly with food. Continue for 3 months after Hb reaches 10.
- If pain after delivery: give **paracetamol** 1g 4-6 hourly (up to 4g in 24 hours) and **ibuprofen**³ 400mg 8 hourly with food for up to 5 days.
- If HIV positive mother not on ART, start ART same day ↗ 113, especially if breastfeeding.

- Treat the HIV-exposed baby → 166.**
- Routinely review mother and baby 6 hours, 6 days, and 6 weeks after delivery.**

¹If possible, avoid taking iron within 4 hours of taking methyldopa and within 2 hours of milk and tea. If on dolutegravir and taking at same time as iron, take with food ²Abdominal pain, nausea, vomiting, constipation. ³Avoid ibuprofen if pre-eclampsia, peptic ulcer, asthma, hypertension, heart failure, kidney disease.